

Patient: [Redacted]

Referring Physician:

[Redacted]

DOB: [Redacted] Age: [Redacted] Gender: F

Ref#: [Redacted] Hosp#: [Redacted] Provider Group: [Redacted]

Date of Service: [Redacted] Date Received: [Redacted] Outpatient
Room: [Redacted] Bed: [Redacted]

UUID: 1A0D203A-BC4B-441F-97F6-D1FF7E959262
TCGA-AC-A3TN-01A-PR

Redacted



FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

A., C, D.) RIGHT BREAST AND AXILLARY SENTINEL LYMPH NODE, MASTECTOMY AND SENTINEL LYMPH NODE BIOPSY:

- Multicentric invasive lobular carcinoma, Nottingham grade 1-2.
 - The area involved by invasive carcinoma measures approximately 8 cm in diameter.
 - See comment.
- Two axillary lymph nodes, no tumor present (0/2).
 - This includes one sentinel lymph node and one non-sentinel lymph node.
- Resection margins are free of tumor. Invasive carcinoma is at least 1 cm from all margins. The closest margin is the inferior, which is approximately 1 cm from carcinoma.
- Lobular carcinoma in situ (LCIS), classic type, low nuclear grade, widespread throughout the right breast.
- Sites of previous biopsy identified.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
IPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case(s) (circle):	QUALIFIED	DISQUALIFIED
Reviewed (initials)	3/30/12	

PATHOLOGIC TUMOR STAGING SYNOPSIS:

Type and grade (invasive): Invasive lobular carcinoma, Nottingham grade 1.

Type and grade (in situ): Lobular carcinoma in situ, low nuclear grade.

Primary tumor: pT3(m)

Regional lymph nodes: pN0(i-)(sn).

Distant metastasis: Not applicable.

Pathologic stage: IIB.

Lymphovascular invasion: Not identified.

Margin status: Negative (R0).

ICD-03
carcinoma, lobular, infiltrating
8520/3
Site: breast, NOS c50.9

4-3-12
R0

COMMENT: In addition to the two areas of invasive lobular carcinoma previously biopsied, there is a large area of invasive lobular carcinoma in roughly the center of the breast that corresponds to the abnormal area detected in the right breast MRI dated [Redacted] (the suspicious area measured 8.1 cm in diameter on MRI). Grossly, this was an ill-defined area relatively denser than the surrounding breast tissue.

B. LEFT BREAST, PROPHYLACTIC MASTECTOMY:

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- Focal atypical ductal hyperplasia (ADH, in lower outer quadrant).
- Subareolar duct ectasia.
- Focal fibrosis, cysts and mild usual type ductal hyperplasia.
- Discrete focus of scar tissue in central inferior breast.

Breast Invasive Tumor Staging Information

(AJCC Cancer Staging Handbook, [REDACTED], and CAP protocol,

This staging also incorporates:

Previous biopsy: [REDACTED]

Breast profile: [REDACTED]

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Specimen type:
Specimen procedure:
Lymph node sampling:
Specimen integrity:
Specimen laterality:
Specimen size:

Total breast.
Mastectomy.
Sentinel lymph node.
Intact specimen.
Right.
24.4 x 21.8 x 6.4 cm.

INVASIVE TUMOR FEATURES:

Invasive tumor size:
Invasive tumor site:

Invasive tumor focality:
Histologic type:
Total Nottingham Grade:
Tubule formation:
Nuclear Pleomorphism:
Mitotic count for Nottingham:
Mitotic count:
Lymphatic invasion:

Approximately 8 cm in diameter
Central, in addition to areas previously biopsied
(12:00 and 9:30 aspects of right breast).
Multicentric
Invasive lobular carcinoma.
1 of 3.
3 of 3.
1-2 of 3.
1 of 3.
1 mitosis in 10 high power fields.
Not identified.

MARGIN STATUS FOR INVASIVE COMPONENT:

Distance of tumor from margins:
Closest margin:
Other margins:

Negative.
Approximately 1 cm.
Inferior
All other margins at least 1 cm from tumor.

DUCTAL CARCINOMA IN-SITU (DCIS):
LOBULAR CARCINOMA IN-SITU (LCIS):

Absent.
Present, extensive.

Skin:
Nipple:

Unremarkable.
Focal subareolar duct ectasia. Subareolar foci of
invasive lobular carcinoma present.
Not applicable.

Skeletal Muscle:

INVASIVE PATHOLOGIC TUMOR STAGING (pTNM)

Primary tumor (pT):
Regional lymph nodes (pN):
Distant metastasis (pM):
Tumor Stage:

pT3
pN0(i-)(sn).
Not applicable.
IIB

RECEPTOR STATUS AND HER2/NEU:

Estrogen receptors:
Progesterone receptors:
Her2/neu:

Biomarkers performed on
two separate core needle biopsies.
95%.
80% and 50%, respectively.
1+.

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Ki-67 proliferative index:

1-2%.

Signed by _____

Source of Specimen:

- A. Sentinel lymph node; Sentinel node #1, Right breast
- B. Breast; Left
- C. Breast; Right breast-additional inferior margin
- D. Breast; Right

Clinical History/Operative Dx:

Neoplasm of right breast.

Intraoperative Diagnosis:

- A. Sentinel node #1 right breast: touch prep A diagnosis: Negative for carcinoma. (Dr. _____). The intraoperative interpretation(s) was/were performed and rendered at _____.

Gross Description:

A. The specimen is labeled sentinel node #1 right breast and is received without fixative. It consists of a 0.5 x 0.5 x 0.4 cm lymph node. It is serially sectioned and touch imprints are obtained. The node is submitted for permanent section in cassette A1.

B. The specimen is labeled left breast tissue and is received in formalin. It consists of a mastectomy specimen which weighs 1,258 grams. A black suture marks lateral. With this orientation, the specimen

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measures 20.5 cm from superior to inferior, 29 cm from medial to lateral, and 4.5 cm from superficial to deep. There is a broad overlying semi-circular portion of tan skin measuring 23 x 14.5 cm. In the medial inferior edge of the skin, there is a 4.5 cm areola with a protuberant 1.4 cm nipple. The skin is otherwise unremarkable. The anterior-superior margin is inked blue, the anterior-inferior margin is inked green, and the posterior margin, which partially consists of smooth facial appearing tissue, is inked black. The specimen is serially sectioned at close intervals to reveal lobulated fatty tissue and scattered tan-white fibrous parenchyma. In the central inferior portion of the breast, along the superior medial edge of the areola, there is an area of fibrous parenchyma which has a slightly stellate appearance but is soft and rubbery. This stellate area is 2 cm in maximum dimension. It is 4 cm or greater from the deep, inferior, and superior margin and is 2 cm from the closest skin. The breast parenchyma in the lower outer quadrant has a rough fine nodular feel but there are no areas which have a stellate or retracted appearance. There are no lymph nodes identified within the lateral portion of the excision. Representative sections are submitted. Section summary:

- B1) nipple and tissue just deep to nipple,
- B2-B3) stellate fibrous parenchyma central inferior breast,
- B4) representative upper inner quadrant,
- B5) lower inner quadrant,
- B6) upper outer quadrant,
- B7-B8) lower outer quadrant, including representative lower outer quadrant deep margin.

C. The specimen is labeled right breast tissue, additional inferior margin and is received in formalin. It consists of two irregular fragments of lobulated fatty tissue with an aggregate weight of 52 grams. The larger specimen measures 10.5 x 4.8 x 1.6 cm and the smaller specimen measures 4.2 x 2.8 x 1.7 cm. The surgical margins of these specimens are not designated. The larger specimen is inked black and the smaller specimen is inked green. Serial sections of both specimens reveal predominantly lobulated soft to pale yellow fatty tissue without palpable areas of nodularity. Representative sections of the larger specimen are submitted in cassettes C1-C3. Representative sections of the smaller specimen are submitted in cassette C4. Following review of the initial slides, additional sections are submitted in C5-C9.

D. The specimen is labeled right breast tissue and is received without fixative. It consists of a mastectomy specimen weighing 1,285 grams. A black suture is present and is not otherwise designated but is arbitrarily assumed to represent lateral. With this orientation, the specimen measures 24.4 cm from medial to lateral, 21.8 cm from superior to inferior, and 6.4 cm from superficial to deep. There is an overlying broad elliptical portion of tan skin which measures 24 x 16 cm. In the medial inferior skin, there is a 4 cm areola and 1.5 cm protuberant nipple. The skin shows two small areas of hemorrhagic discoloration which are located 3 and 3.5 cm superior to the nipple. No other skin lesions are identified. The anterior-superior margin is inked blue, the anterior-inferior margin is inked green, and the posterior margin, which consists partially of smooth facial tissue, is inked black.

The breast is serially sectioned at close intervals to reveal a firm stellate appearing tumor mass within the central lateral breast, approximately 4.5 cm lateral to the nipple. This stellate area is 1.2 cm in maximum dimension. Centrally, within this stellate mass is a biopsy site clip. This stellate mass is 3.5 cm from the closest inferior margin, 11 cm from the closest superior margin, 14 cm from the closest medial margin, and 11 cm from the closest lateral margin. It is 3.5 cm from the closest deep margin and approximately 3 cm

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from the skin. A second palpably firm and stellate appearing mass is located in the superior central breast approximately 4 cm superior to the nipple. This second area measures 0.8 cm in maximum dimension. It is 4 cm from the closest deep margin, 3.5 cm from the closest skin, 6.5 cm from the closest inferior margin, 9 cm from the closest superior margin, 10 cm from the closest medial margin, and greater than 10 cm from the lateral margin. A twisted biopsy site clip is also embedded within this stellate mass.

The central breast tissue has an ill-defined firm area measuring approximately 7-8 cm in diameter. This area of firmness involves multiple quadrants of the breast. In the far lateral portion of the breast, there is a firm 1.8 cm lymph node.

Representative tissue of the central lateral stellate mass is retained for research purposes. Representative sections are submitted. Section summary:

- D1) nipple and tissue just deep to nipple,
- D2-D3) complete cross-section of central lateral palpable mass,
- D4) additional section of superior edge of central lateral tumor,
- D5-D6) sections of central superior tumor mass (biopsy site clip in cassette D5),
- D7) central superior deep margin,
- D8) central lateral deep margin,
- D9) representative upper inner quadrant,
- D10) representative medial lower inner quadrant,
- D11) representative lateral lower inner quadrant,
- D12) representative lateral lower inner quadrant and closest anterior inferior margin.
- D13) upper outer quadrant,
- D14) lower outer quadrant,
- D15-D16) lymph node lateral portion of excision.

Microscopic Description:

A. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered. Immunohistochemical stain: , showing no evidence of metastatic carcinoma. Appropriate positive and negative controls reviewed.

B. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

C. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

D. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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